

Digital Community Systems Implementation Guide

A companion to the Digital Community System Toolkit

DESIGN THE RIGHT SYSTEM

Assess, understand users, prioritize needs, set the vision



BUILD IT WELL

Specify, configure, integrate, test, and deploy



SUSTAIN AND IMPROVE

Operate, monitor, learn, govern change, and scale

The Data.FI DCS package links person-centered planning to standards-based implementation and continuous improvement.

Purpose of this guide

This guide helps country teams translate the decisions produced through the Digital Community System Toolkit into a governed, configured, connected, tested, and supportable digital community system. The Data.FI reference eCHIS provides the primary worked example and a reusable technical foundation.

Document information	Details
Project	Data.FI
Companion resource	Digital Community System Toolkit: Resources for Developing Human-Centered Digital Tools to Support Delivery of Integrated Community Services
Primary audience	Government product owners, community health programs, digital health authorities, implementation partners, architects, interoperability teams, data teams, administrators, and support teams
Status	Internal working draft for technical and editorial review

Contents

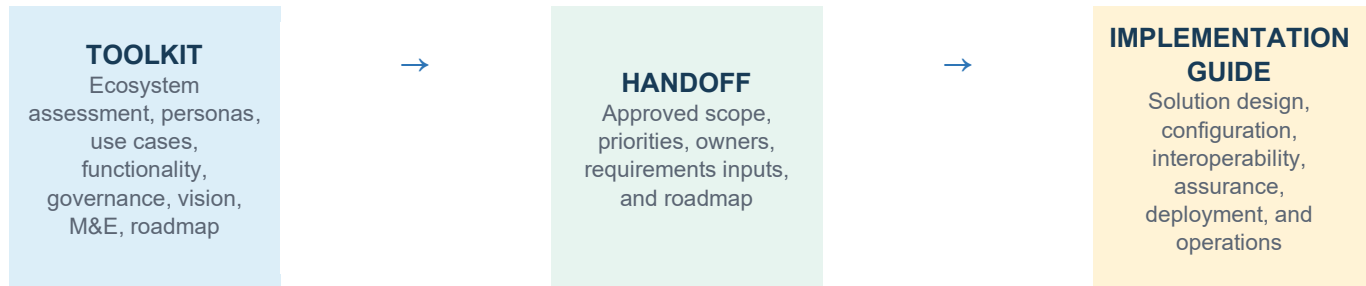
This outline is provided for internal circulation and feedback. Page numbers can be refreshed in Word after final pagination.

Section	Contents
How the package fits together	Relationship to the DCS Toolkit, entry points, and guide map
Executive summary	Purpose, reference implementation, country ownership, and traceability
Part I. Mobilize implementation	1. Confirm implementation inputs; 2. Establish governance; 3. Define the pathway
Part II. Translate priorities into solution design	4. Connect personas and use cases; 5. Document workflows; 6. Develop requirements; 7. Define releases
Illustrative clinical and service workflows	8. Workflow package method; 9. Registration and household enrollment; 10. Scheduled community visit; 11. Maternal and newborn continuity; 12. Child health and immunization; 13. Community surveillance; 14. Commodity management; 15. Referral and counter-referral
Part III. Configure the digital community system	16. Reference architecture; 17. FHIR contract; 18. Configuration; 19. Users and organizations; 20. Infrastructure
Part IV. Connect the ecosystem	21. Interoperability method; 22. Reference map; 23. Reusable patterns; 24. Interface specifications; 25. Production hardening
Illustrative integration workflow specifications	26. Identity reconciliation; 27. Community referral; 28. Counter-referral; 29. Supply issue and receipt; 30. Surveillance and alerts; 31. Routine reporting; 32. Analytics ingestion
Part V. Verify, deploy, and institutionalize	33. Assurance; 34. Go-live; 35. Training; 36. Deployment waves; 37. Operations; 38. Sustainability
Part VI. Reusable implementation assets	39. Country package; 40. Core templates; 41. Content ownership; 42. Companion resources
Annexes proposed for the next draft	A. Workflow catalog; B. Data dictionary; C. Decision logic; D. Indicators; E. Requirements; F. Interface catalog; G. Mappings; H. Test cases; I. Runbooks

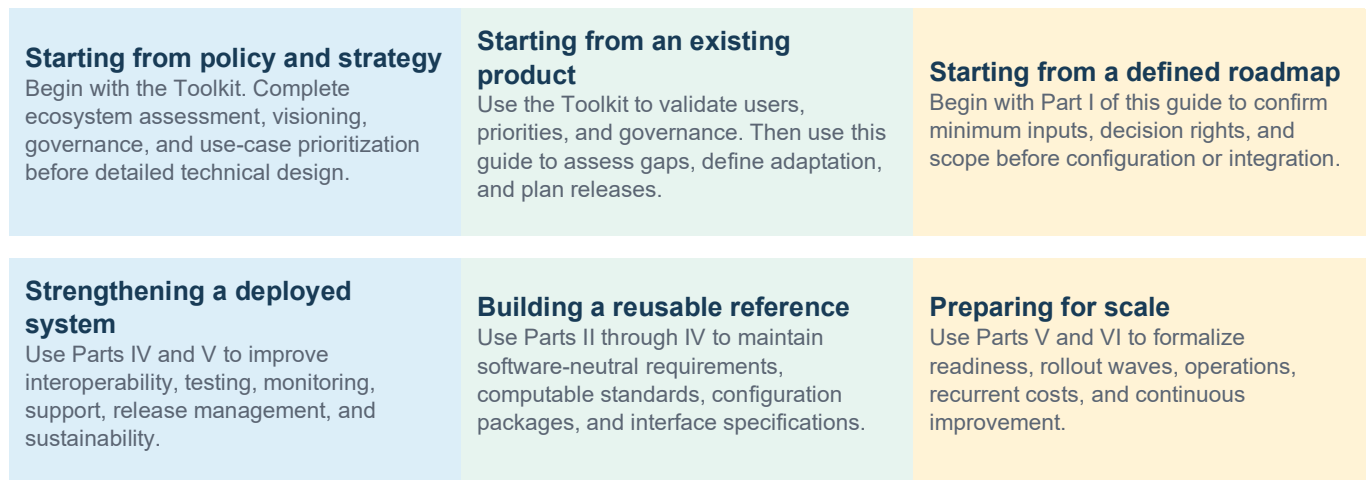
How the package fits together

The Digital Community System Toolkit and this Implementation Guide form one connected package. The Toolkit supports collaborative planning and person-centered

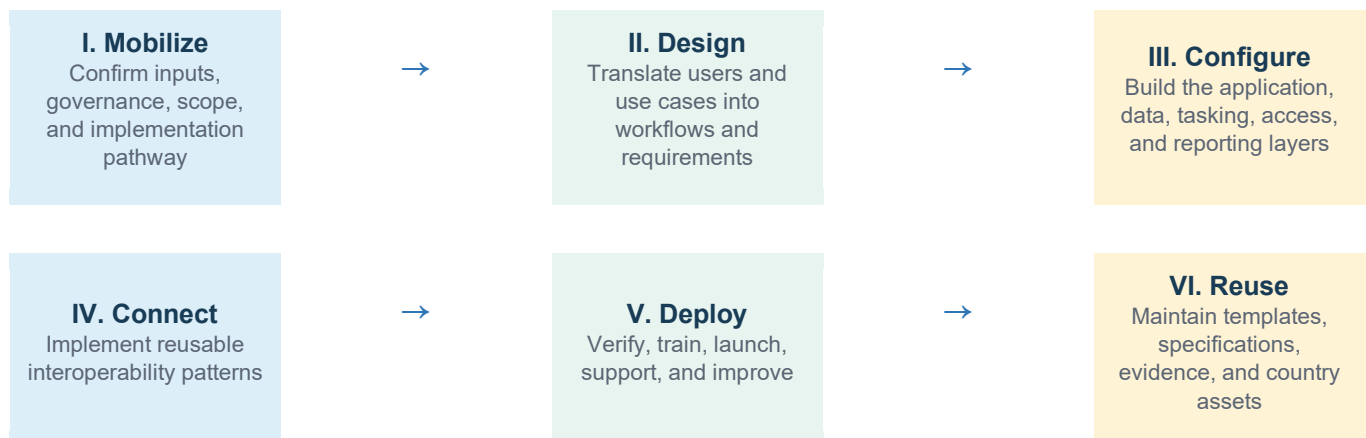
design. This guide begins when a country is ready to turn those decisions into implementable specifications, configuration, integrations, deployment, and long-term operations.



Choose the entry point that matches country readiness



Guide map



Executive summary

Digital community systems support coordinated service delivery across households, communities, facilities, programs, and national information systems. Effective implementation starts with a shared vision and a clear understanding of users. It continues through disciplined requirements, standards-based configuration, governed interoperability, rigorous testing, prepared users, and sustainable operations.

The Data.FI reference eCHIS demonstrates one implementation of this approach. It is configuration-driven and offline-first. A community health worker completes a Questionnaire; template extraction creates structured FHIR records and, when required, the next Task. The reference model uses daisy-chained tasks for scheduling, aligns selected records with IPS concepts, and applies selective synchronization so devices receive the records needed for frontline work while other records can support analytics.

This guide uses that implementation as a worked example. Country teams may adopt the full reference product, adapt selected components, or reuse specifications and integration patterns with another platform. The implementation process remains country-led. Responsible authorities approve service content, workflows, terminology, identifiers, access, architecture, hosting, exchange, reporting, privacy, security, and operations.

Implementation principle

Maintain traceability from each approved user need to a workflow, requirement, configuration or code asset, test result, release decision, and operational measure. This creates a clear line from person-centered design to accountable implementation.

Part I. Mobilize implementation

1. Confirm the implementation inputs

Before technical work begins, confirm that planning outputs are sufficiently mature to guide implementation. Gaps can be resolved through focused design sessions rather than reopening the full planning process.



Minimum inputs from the Digital Community System Toolkit

1.1 Readiness questions

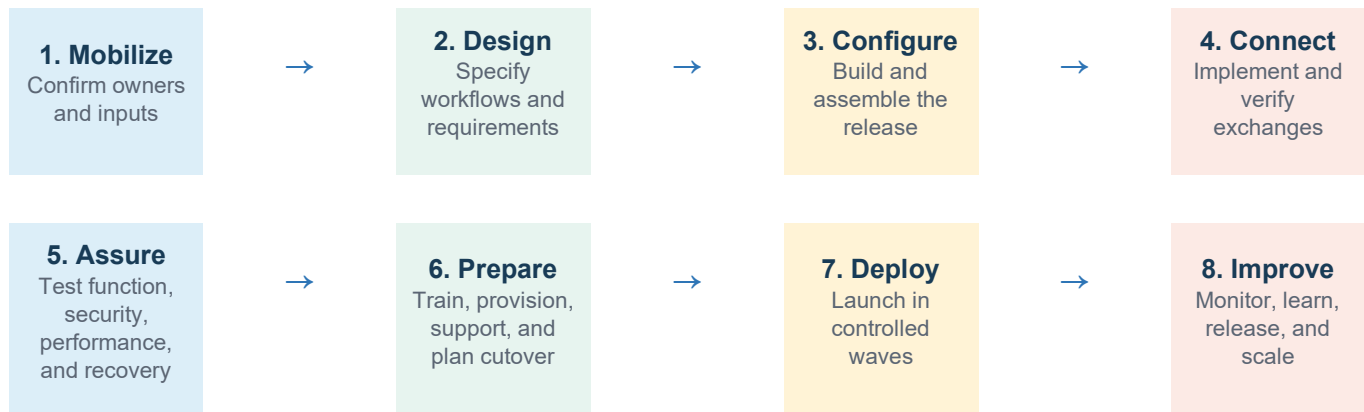
- Is there a named government product owner with authority to prioritize scope and accept releases?
- Are priority users and service-delivery workflows documented and validated?
- Are the first-release use cases explicit and feasible within available resources?
- Are relevant policies, standards, data-protection requirements, and architecture constraints identified?
- Are source systems, systems of record, identifiers, reporting obligations, and integration owners known?
- Are implementation, operations, support, and recurrent-cost responsibilities assigned?

2. Establish implementation governance

Implementation governance should connect national leadership, product decisions, and technical delivery. The structure must resolve clinical, program, data, architecture, security, operational, and funding questions without leaving delivery teams to make policy decisions by default.



3. Define the implementation pathway

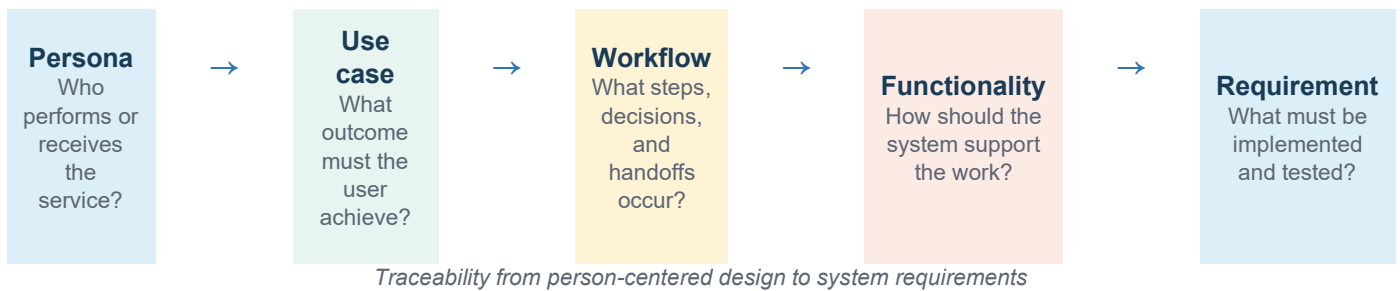


The pathway is iterative. Teams may return to design when testing or field use reveals a gap. Each iteration should preserve the approved requirements, decision log, configuration inventory, test evidence, and release history.

Part II. Translate priorities into solution design

4. Connect personas, use cases, and system behavior

Use the personas and prioritized use cases developed through the Toolkit as the basis for implementation. Do not begin with screens or products. Begin with the user outcome, operating context, service workflow, and information needed to complete the work.



5. Document service workflows

Each priority workflow should be documented in a common format. The narrative should be readable by program and service-delivery teams. The detailed model should be precise enough for configuration, integration, and testing.

Trigger and eligibility Define what starts the workflow, who qualifies, and what information must already exist.	Actors and locations Identify who initiates, performs, reviews, receives, approves, or follows up, and where work occurs.	Service steps Describe actions in service order, including offline work, handoffs, and information shown to the user.	Decisions and rules Record conditional paths, calculations, validation, alerts, escalation, and decision support.
Records and terminology Define data captured or reused, identifiers, code systems, minimum data, and source of truth.	Tasks and follow-up Specify scheduling, priority, due windows, reassignment, successor tasks, closure, and overdue behavior.	External exchange Identify systems, direction, payload, timing, acknowledgment, return flow, and reconciliation.	Completion and measures Define the completion state, user confirmation, indicators, acceptance criteria, and monitoring.

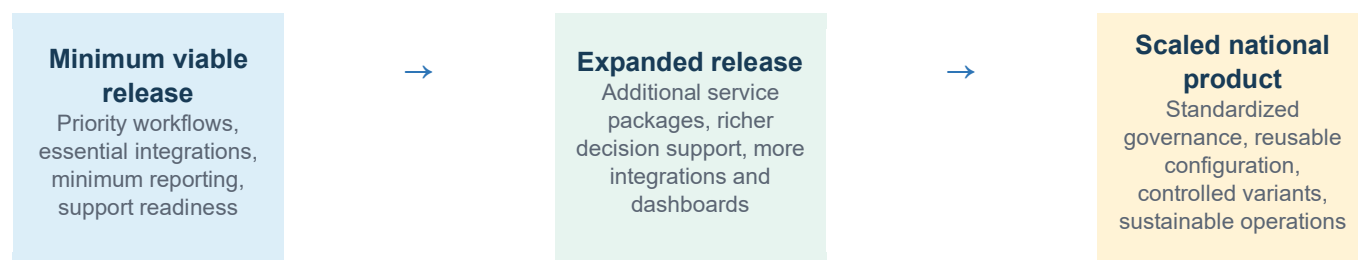
6. Develop requirements and acceptance criteria

Translate each workflow into functional and non-functional requirements. Functional requirements describe the system behavior needed to support users. Non-functional requirements establish the conditions under which the solution must operate, including offline use, performance, security, privacy, scalability, accessibility, maintainability, and supportability.

Recommended requirement form

The system shall [perform a specific behavior] for [named user or system] when [condition or trigger] so that [service outcome]. Acceptance criteria should define observable evidence, including positive, negative, offline, synchronization, and error-recovery scenarios.

7. Define the product scope and release model



Maintain one product backlog. Label requirements by release and link them to source use cases. Country-specific requests should be assessed for policy alignment, user value, reuse potential, interoperability impact, cost, maintenance, and testing needs.

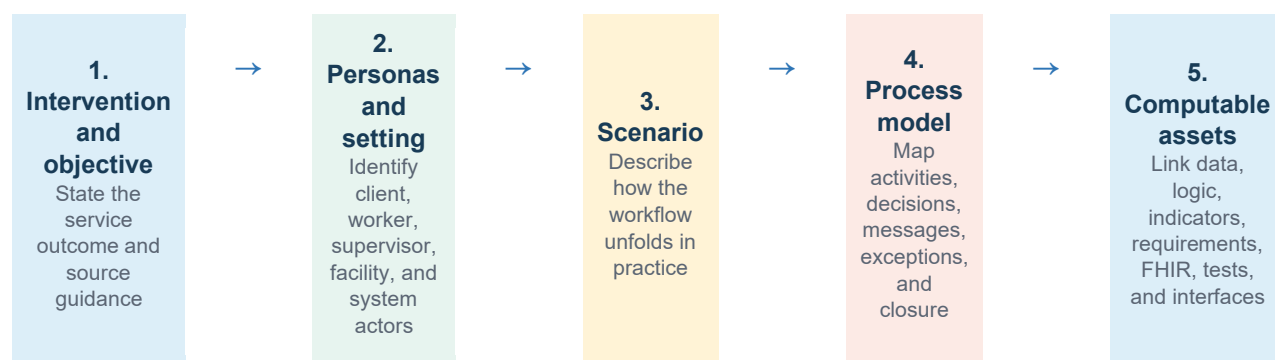
Illustrative clinical and service workflows

This section applies the operational structure used in WHO digital adaptation kits to the Data.FI digital community systems reference. Each workflow is presented as a generic, software-neutral starting point and should be adapted to national service-delivery guidance, scopes of practice, terminology, reporting obligations, and referral arrangements. The detailed workflow package should link the narrative, activities, data elements, decision-support logic, indicators, requirements, FHIR artifacts, and tests with stable identifiers.

Workflow notation and traceability

Use DCS.[domain].[process] for each workflow, DCS.[domain].[process].[activity] for activities, DE for data elements, DT for decision tables, IND for indicators, REQ for functional requirements, and INT for interface requirements. Example: DCS.MNCH.ANC.04 is an ANC activity, DCS.MNCH.ANC.04.DE.01 is a linked data element, and DCS.MNCH.ANC.DT.03 is a decision rule used at that point.

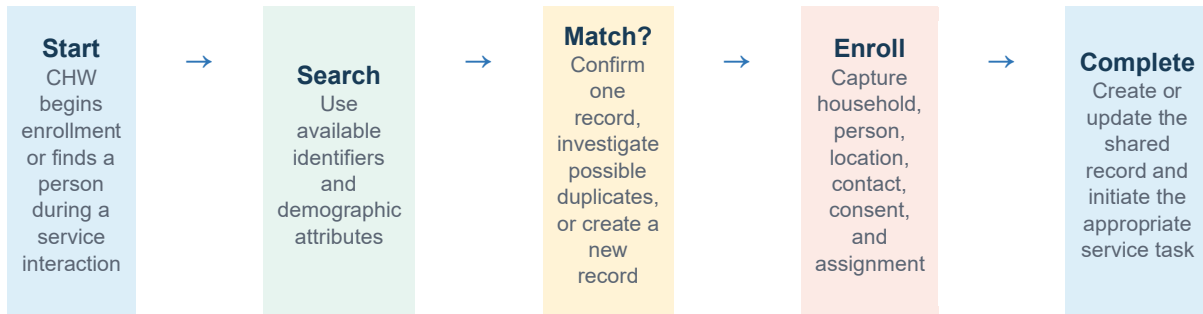
8. Build a complete workflow package



Workflow field	Required content
Process header	Process name and ID, objective, trigger, end state, primary persona, supporting actors, locations, offline requirement, and linked guidance.
Activity annotations	Purpose, responsible actor, inputs, user action, system action, outputs, exceptions, data created or reused, and next activity.
Decision support	Decision ID, trigger, inputs, business rule, output, action, annotation, and approval source.
Data	Data-element ID, label, definition, type, allowed values, validation, required/conditional status, terminology mapping, FHIR path, sensitivity, and indicator linkage.
Requirements	User story, acceptance criteria, offline and sync behavior, access rule, audit requirement, and release.
Assurance	Positive, negative, boundary, offline, synchronization, duplicate, referral, and recovery tests.

9. Registration and household enrollment

Objective: locate an existing person and household record or create a new record, confirm identity and consent, assign the correct geographic and organizational context, and prepare the person for service delivery.



Key personas

Community member, community health worker, supervisor, registry or MPI service.

Key decisions

Existing record? Duplicate suspected? Consent granted? Person belongs to the assigned catchment?

Core outputs

Person and household record, identifiers, relationships, location assignment, care-team assignment, registration status.

System behavior

Support offline search scope, temporary local identifiers, validation, duplicate warnings, and later reconciliation.

Integration touchpoints

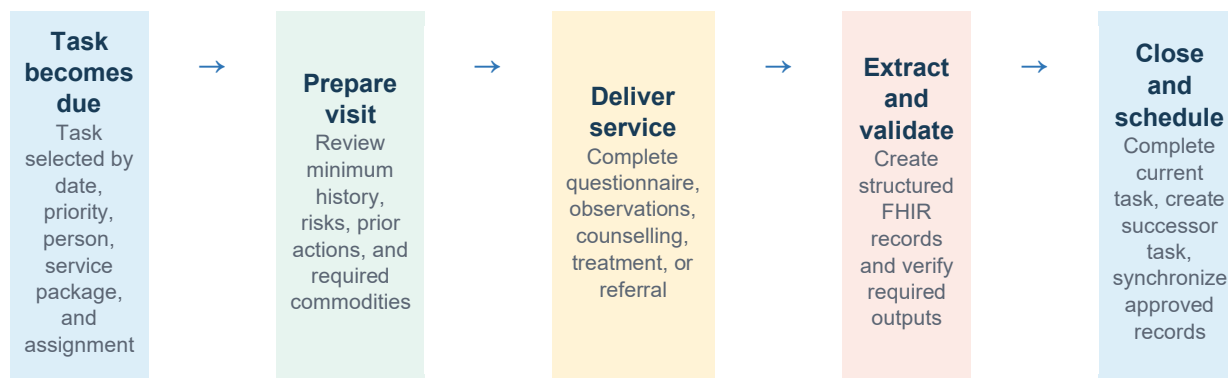
Master patient index, facility registry, geographic hierarchy, identity service.

Assurance focus

No silent duplicates; no record exposure outside user scope; deterministic reconciliation after sync.

10. Scheduled community visit

Objective: help the community health worker identify due work, complete the appropriate service form, record structured outputs, and create the next follow-up action.



Illustrative data

Task status, encounter date, observations, service provided, counselling, commodity use, referral, follow-up date.

Decision logic

Eligibility, danger signs, required referral, task timing, overdue handling, and next-service calculation.

Offline behavior

Show locally available history, save complete transaction locally, prevent double submission, queue sync.

Measures

Due tasks completed, overdue tasks, successful sync, service coverage, referral initiated, data completeness.

Exceptions

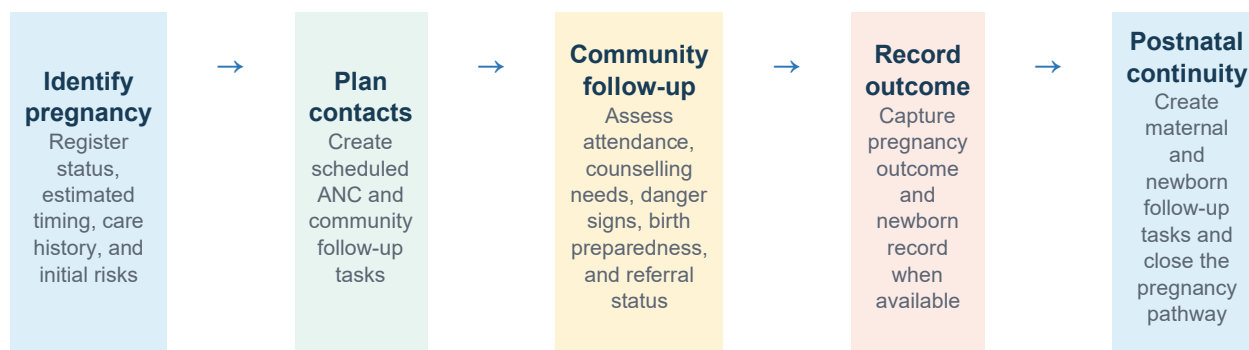
Person moved, unavailable, deceased, duplicate task, stock unavailable, device reassigned, task cancelled.

Linked assets

Questionnaire, extraction template, Task profile, terminology, acceptance tests, monitoring rules.

11. Maternal and newborn continuity workflow

Objective: maintain continuity from pregnancy identification through antenatal contacts, birth outcome, postnatal follow-up, danger-sign referral, and transition to routine maternal and child services. This is an illustrative community workflow, not a clinical guideline. Countries should populate its clinical content from approved national and WHO guidance.



Activity group

Illustrative specification

Pregnancy identification	Confirm person record; capture pregnancy status and timing; record source; evaluate immediate referral trigger; link to existing facility record when available.
ANC continuity	Track scheduled and completed contacts; surface missed care; record community counselling and referral; avoid duplicating facility clinical records.
Danger signs	Invoke a country-approved decision table; display urgent action; create referral record; support contact with facility; retain escalation and acknowledgment.
Birth outcome	Capture minimum outcome data from an authorized source; create or link newborn; update maternal status; record deaths and transfers under approved rules.
PNC follow-up	Schedule maternal and newborn tasks; assess continuity, referral closure, immunization linkage, breastfeeding support, and unresolved risks.

12. Child health and immunization workflow

Objective: identify children due for routine services, review available history, support community-level screening and counselling, record services within scope, refer when required, and schedule the next contact.

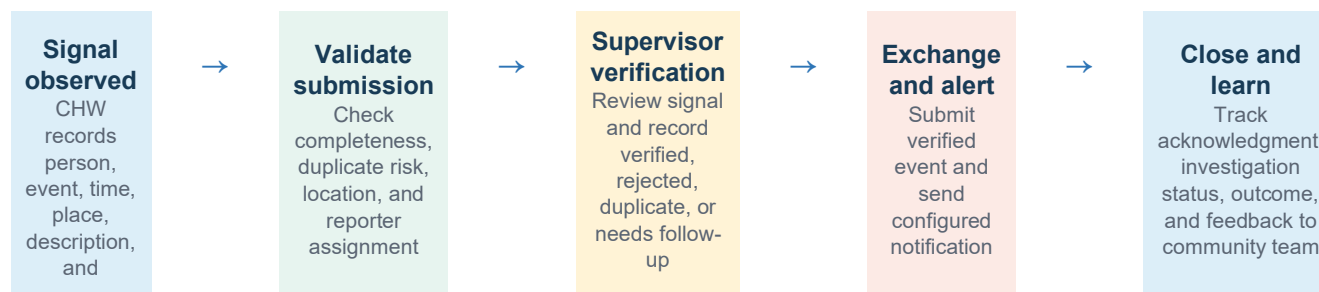


Clinical content boundary

The implementation guide should specify where the system invokes approved clinical content and decision logic. It should not invent clinical rules. Each rule should identify its source, approving authority, version, effective date, and linked test cases.

13. Community event-based surveillance

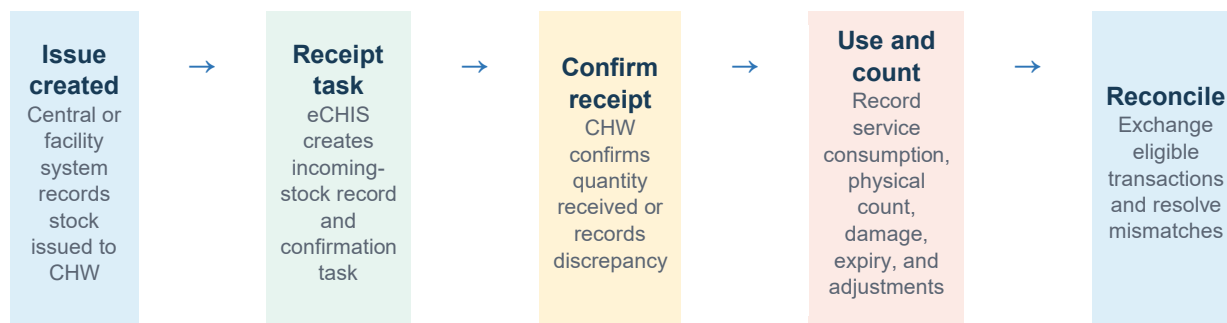
Objective: capture a community signal, apply an approved verification process, escalate qualifying events, exchange the minimum required information with the surveillance platform, and maintain closure status.



immediate
action

14. Community commodity management

Objective: maintain an accountable view of commodities held by community workers, confirm issues and receipts, record consumption and adjustments, and exchange approved stock transactions with the logistics system.



15. Referral and counter-referral

Objective: move a person from community service to an appropriate facility or service, transmit the minimum referral information, receive the service outcome, and create a community follow-up task.



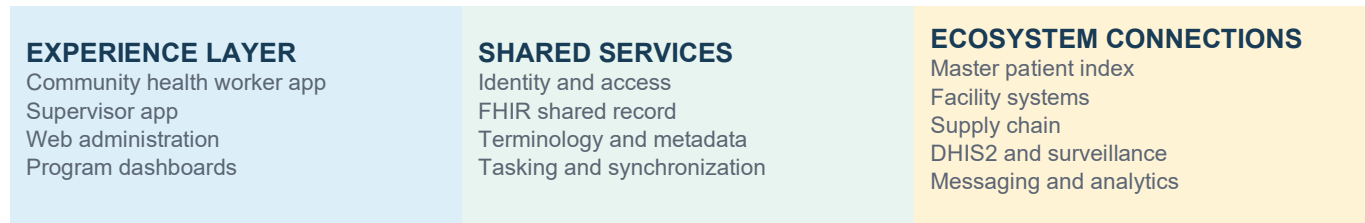
Country adaptation questions

Which cadres may initiate each referral? Which referral reasons and urgency categories apply? What information may cross organizational boundaries? Which system owns referral status? What confirms receipt, service completion, and closure? How are failures, duplicates, rejected referrals, and people who do not arrive handled?

Part III. Configure the digital community system

8. Reference solution architecture

The Data.FI reference solution combines mobile applications, web administration, identity and access management, a FHIR server, integration orchestration, analytics pipelines, a data warehouse, dashboards, and platform monitoring. It can connect to identity, facility, logistics, surveillance, messaging, clinical, and national reporting systems.



Cross-cutting foundation

Governance, privacy, security, infrastructure, monitoring, backup and recovery, release management, documentation, and support apply across every architecture layer.

9. Use the FHIR Implementation Guide as the computable contract

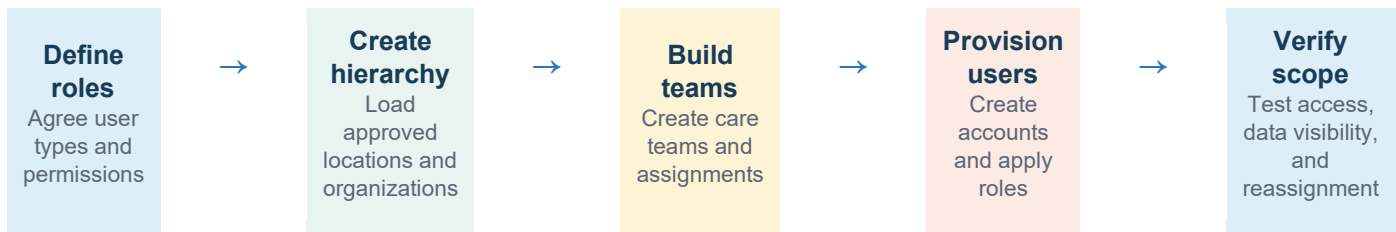
The Data.FI Reference eCHIS Implementation Guide describes the FHIR resources produced by the reference application. It should serve as the authoritative source for profiles, extensions, value sets, code systems, search parameters, examples, and design notes. This narrative guide explains implementation decisions. The FHIR Implementation Guide records the machine-testable result.



10. Configure content and behavior

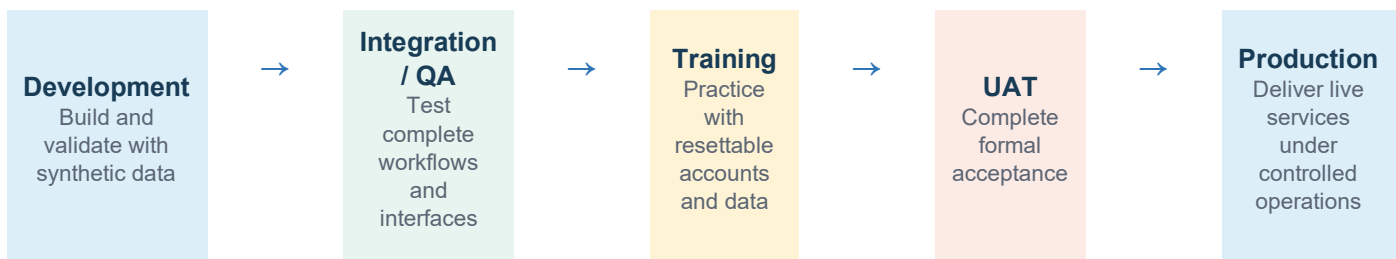
Application experience Menus, labels, navigation, feature flags, language, accessibility, device settings, and user feedback.	Forms and rules Questionnaires, validation, conditional display, calculations, decision support, and extraction logic.	Terminology Code systems, value sets, concept maps, approved displays, translations, and terminology ownership.	People and access Users, groups, roles, permissions, authentication, service accounts, and privileged access.
Organizations and locations Administrative hierarchy, facilities, community units, organizations, care teams, and assignments.	Tasking and scheduling Task types, priorities, due rules, successor logic, status transitions, reminders, and exceptions.	Reporting and analytics Indicators, warehouse models, refresh schedules, exports, dashboards, and reconciliation.	Integration settings Endpoints, metadata, credentials, triggers, cursors, retries, idempotency, and monitoring.

11. Manage users, organizations, and locations



12. Plan environments and infrastructure

Separate environments protect production data and create stable spaces for development, integration testing, training, and acceptance. Capacity planning should consider expected users, households, records, forms, sync patterns, integrations, analytics load, growth, availability, backup, and recovery targets.



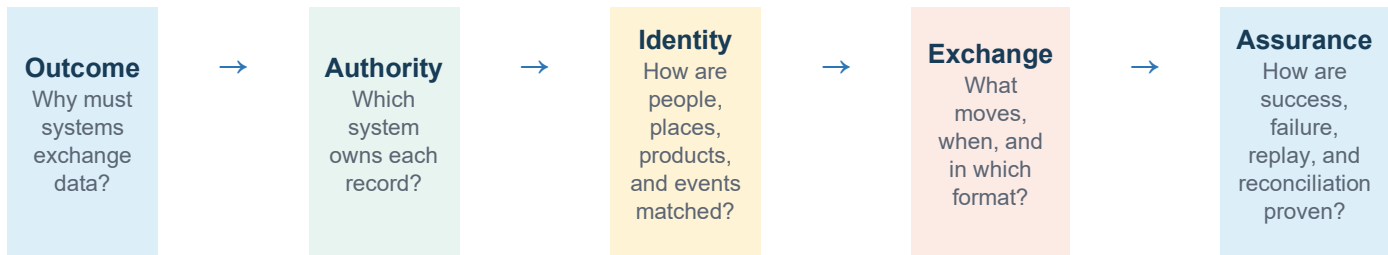
Infrastructure decision record

Document hosting model, network zones, platform components, database, storage, backup, recovery, observability, certificates, domains, scaling assumptions, support ownership, data residency, and recurrent costs.

Part IV. Connect the ecosystem

13. Design interoperability around service outcomes

Treat each interface as a business workflow. Start with the service or reporting outcome. Identify systems of record, stable identifiers, minimum data, terminology, security, timing, acknowledgments, return flows, retries, and reconciliation before writing integration code.



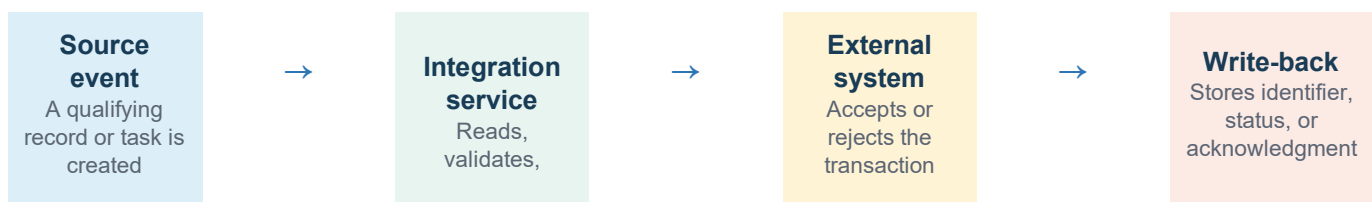
14. Reference interoperability map



Figure: Reference connection patterns around the shared community record.

15. Use three reusable integration patterns

15.1 Transactional exchange





15.2 Closed-loop service exchange



15.3 Analytical exchange



16. Specify each interface

Every interface should have a versioned specification and operational runbook. Detailed field mappings belong in controlled annexes or repositories rather than the main guide.

Overview Purpose, owners, systems, scope, data classification, status, and version.	Preconditions Metadata, identifiers, permissions, source data, configuration, and dependencies.	Trigger and processing Schedule or webhook, query, cursor, filters, ordered steps, and branches.	Mapping Source path, destination field, type, transformation, terminology, optionality, and sensitivity.
Reliability Idempotency, retries, hold queue, replay, reconciliation, and manual remediation.	Security Transport, authentication, authorization, least-privilege credentials, logging, and retention.	Monitoring Run summary, alerts, dashboard, log review, support ownership, and escalation.	Testing and launch Contract, negative, volume, replay, end-to-end tests, cutover, and rollback.

17. Harden reference workflows for production

Production rule

Reference endpoints, credentials, metadata identifiers, schedules, retention settings, and logging behavior are examples. Replace them with approved country values and retain evidence of security, privacy, program, and operational review.

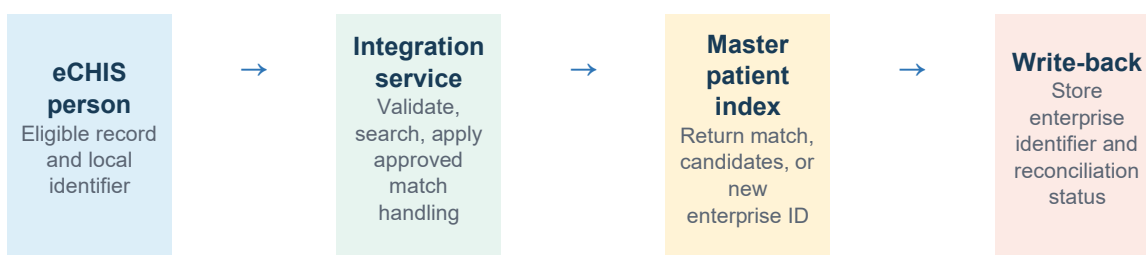
- Confirm every shared field and its approved purpose.
- Apply data residency, retention, consent, and disclosure requirements.
- Use least-privilege credentials and review privileged access.
- Prevent personal data from appearing unnecessarily in logs, notifications, or support tools.
- Validate metadata, terminology, identifiers, and system permissions in the production environment.
- Complete end-to-end, negative, replay, performance, security, and reconciliation testing.
- Establish monitoring, incident response, support ownership, and change control before activation.

Illustrative integration workflow specifications

The following specifications expand the reference integration patterns into reviewable workflow summaries. They are intentionally illustrative. Production implementation requires approved endpoints, metadata, credentials, identifiers, mappings, terminology, data-sharing authority, error handling, monitoring, and test evidence.

26. Identity reconciliation and enterprise identifier write-back

Field	Illustrative specification
Process ID	DCS.INT.ID.01
Outcome	A person in eCHIS is matched to or created in the approved master patient index, and the enterprise identifier is written back to the eCHIS record.
Trigger	New or changed person record becomes eligible for reconciliation.
Source and destination	eCHIS FHIR server → integration service → MPI → eCHIS FHIR server.
Core steps	Read eligible record; validate minimum demographics; search MPI; assess candidate response; create when allowed; write identifier and reconciliation status; retain audit evidence.
Reliability	Use stable source identifier and assigning authority; prevent repeated person creation; hold ambiguous matches; retry transient failures; reconcile write-back failures.
Monitoring	Eligible records, successful matches, new MPI records, ambiguous matches, failed requests, missing write-backs, processing latency.
Tests	Exact match, no match, multiple candidates, missing required data, MPI timeout, duplicate replay, unauthorized write, write-back conflict.



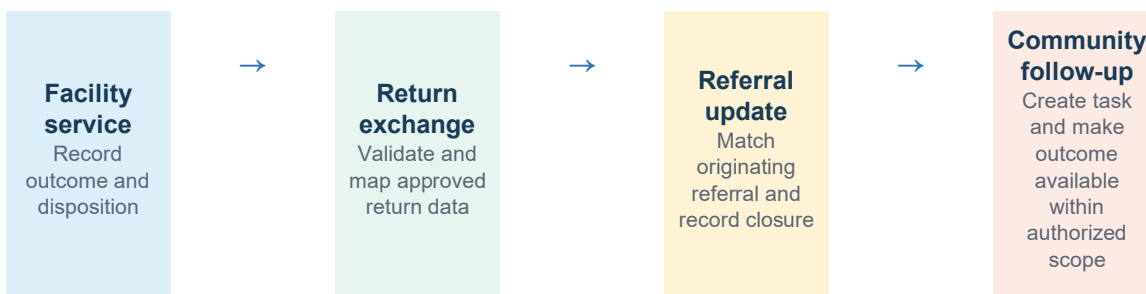
27. Community referral to facility

Field	Illustrative specification
Process ID	DCS.INT.REF.01
Outcome	A community referral and relevant health summary become available to the receiving facility without creating duplicate people or referrals.
Trigger	Approved referral record enters ready-to-send state.
Source and destination	eCHIS → integration service → facility EMR or referral system.
Core steps	Resolve patient identity; identify destination; transform referral and summary; create or update facility referral/order/encounter representation; store destination identifiers and acknowledgment.
Data minimum	Person identifiers; referral reason and urgency; referring actor and location; destination; relevant summary; timestamps; consent or disclosure basis; source record IDs.
Reliability	Idempotent referral key; destination validation; retry and hold queue; duplicate detection; response and write-back; periodic reconciliation.
Tests	Known and unknown patient; destination unavailable; missing metadata; duplicate replay; rejected referral; partial creation; delayed acknowledgment.

28. Facility outcome and counter-referral closure

Field	Illustrative specification
Process ID	DCS.INT.REF.02
Outcome	The community system receives an approved service outcome, updates referral status, and creates any required follow-up task.
Trigger	Facility records a qualifying disposition, service outcome, or closure event.
Source and destination	Facility EMR or referral system → integration service → eCHIS.
Core steps	Read qualifying outcome; identify originating referral and person; transform approved return data; update referral; create closure record and follow-up task; acknowledge processing.

Exceptions	No originating referral, unmatched person, already closed referral, revised facility outcome, missing follow-up timing, disclosure restriction.
Monitoring	Outcomes received, matched referrals, referrals closed, follow-up tasks created, unmatched outcomes, duplicate outcomes, processing failures.



29. Supply issue, receipt confirmation, and stock adjustment

Field	Illustrative specification
Process IDs	DCS.INT.SCM.01 issue-to-receipt; DCS.INT.SCM.02 receipt/adjustment-to-LMIS
Outcome	The CHW can confirm issued stock, and approved receipt, damage, and expiry transactions are reflected in the logistics system.
Triggers	LMIS issue event; CHW receipt confirmation; approved adjustment record.
Identifiers	Commodity, lot or batch when used, issuing and receiving locations, worker or stock point, transaction, reason, and source record.
Sequence	Receive issue; create incoming-stock record and task; confirm quantity; post receipt before downstream adjustments; reconcile accepted and rejected transactions.
Controls	No negative stock without approved rule; prevent duplicate transactions; retain source quantity and destination response; surface discrepancies for review.
Tests	Full and partial receipt, unknown commodity, invalid location, duplicate transaction, adjustment before receipt, expired batch, LMIS rejection, retry.

30. Surveillance and configured alerts

Field	Illustrative specification
Process IDs	DCS.INT.SURV.01 adverse event; DCS.INT.SURV.02 verified community signal
Outcome	Qualifying and verified records are exchanged with the surveillance platform and configured alerts reach approved recipients.

Trigger	Approved event state, such as verified or ready for exchange.
Source and destination	eCHIS → integration service → DHIS2 Tracker or approved surveillance platform; optional messaging service.
Core steps	Validate event and metadata; resolve org unit and tracked entity as required; create or update program record; store destination IDs; send configured alert; retain delivery result.
Privacy	Use minimum necessary alert content; avoid sensitive personal data in messages unless explicitly approved; apply recipient and retention rules.
Tests	New and existing tracked entity, missing org mapping, invalid option code, duplicate event, rejected payload, alert failure, replay, revised verification status.

31. Routine aggregate reporting

Field	Illustrative specification
Process ID	DCS.INT.REP.01
Outcome	Approved aggregate values derived from governed individual-level data are submitted by reporting unit and period.
Trigger	Reporting period closes or approved scheduled run begins.
Source and destination	Governed warehouse or reporting mart → integration service → DHIS2 aggregate or national HMIS.
Core steps	Verify source refresh; calculate values; apply reporting-unit mappings; run data-quality checks; build non-empty payload; submit; retain import summary; reconcile rejected values.
Controls	Version indicator logic; retain numerator/denominator lineage; prevent duplicate period submission; control revisions; require approval where policy dictates.
Tests	No activity, zero value, late records, mapped and unmapped org unit, revised period, rejected data element, partial import, rerun.

32. Analytics ingestion and transformation

Field	Illustrative specification
Process ID	DCS.INT.ANA.01
Outcome	New and changed approved FHIR resources are loaded into governed analytical models for dashboards, indicators, and reporting.
Trigger	Scheduled incremental run or approved event trigger.
Source and destination	HAPI FHIR server → ingestion → raw warehouse → transformations → analytical marts and dashboards.

Core steps	Read by cursor or last-updated boundary; retain source identifiers and versions; upsert raw records; quarantine invalid records; transform into tested models; refresh outputs; publish run summary.
Reliability	Safe overlap window; deterministic upsert; deletion and correction policy; schema-change handling; backfill and replay; lineage from dashboard value to source version.
Monitoring	Resources read, inserted, updated, quarantined, delayed, transformation tests passed/failed, model freshness, dashboard refresh status.

Integration package for each workflow

Maintain a workflow narrative, sequence or swimlane diagram, interface specification, field mapping, terminology mapping, example payloads, test fixtures, security review, deployment configuration, monitoring definition, reconciliation procedure, and operational runbook.

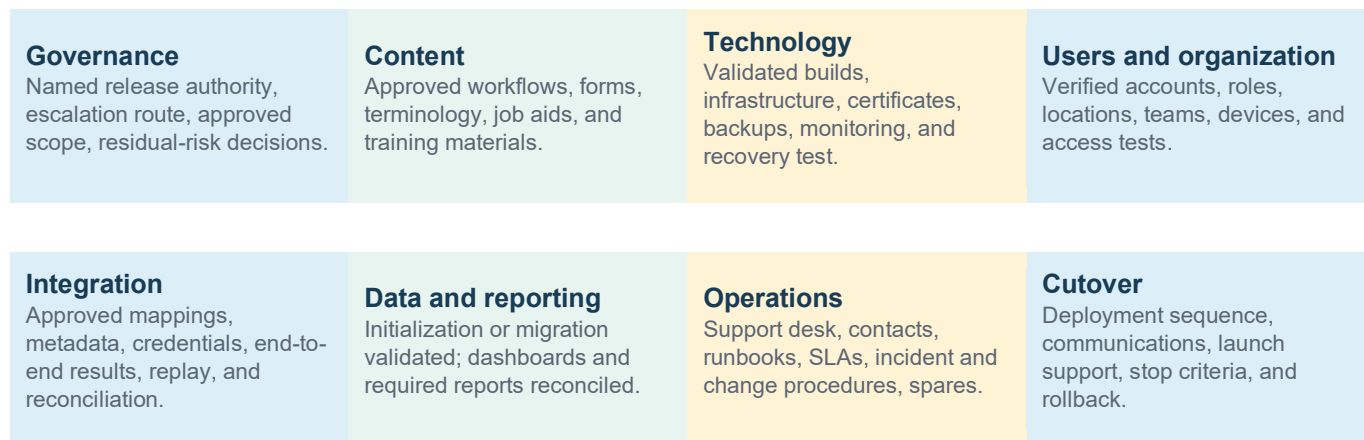
Part V. Verify, deploy, and institutionalize

18. Build assurance into delivery



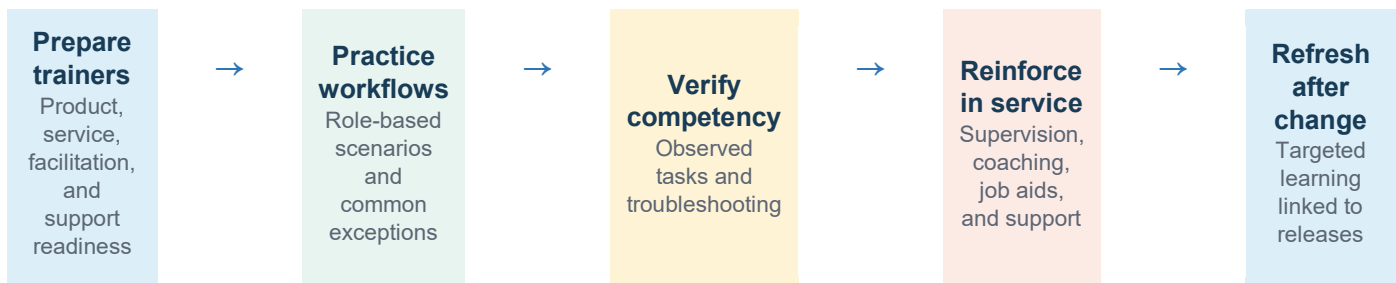
Testing should trace back to approved requirements. Defects should record severity, affected workflow, environment, owner, resolution, retest result, and release decision.

19. Prepare for go-live

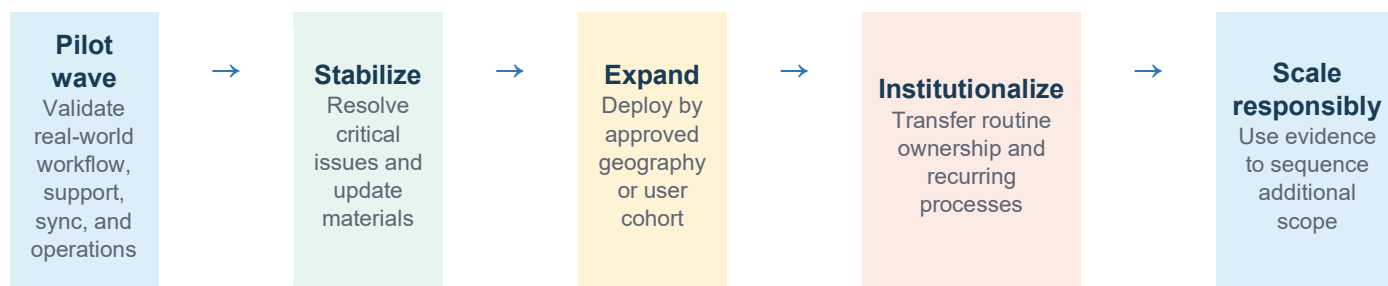


20. Train by role and task

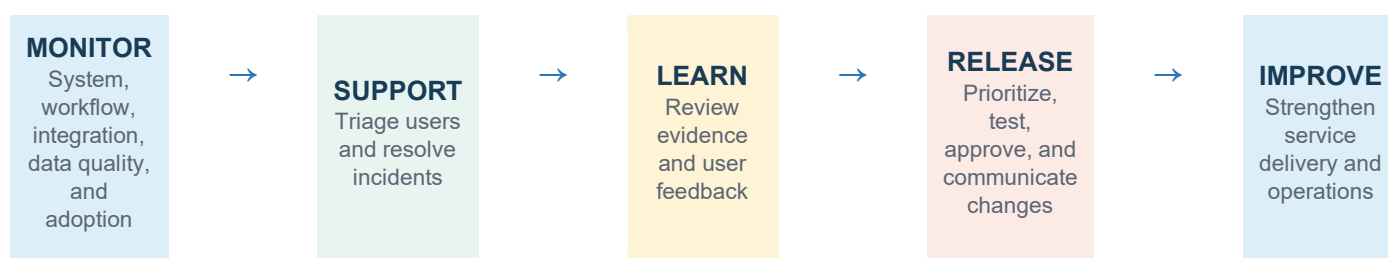
Training should combine service-delivery content and application practice. Prepare supervisors, administrators, and support teams before frontline deployment. Use a distinct training environment and realistic scenarios. Training evidence should include competency checks, not attendance alone.



21. Deploy in controlled waves



22. Operate and improve the product



Continuous controls

Access review, security management, backup and recovery, documentation, vendor management, and cost monitoring.

Product measures

Active use, task completion, sync health, referral closure, data timeliness, integration success, and support performance.

Service measures

Country-approved measures of coverage, quality, continuity, equity, commodity availability, surveillance, and outcomes.

23. Plan sustainability and scale

A sustainable DCS requires institutional ownership, a stable product team, operating capacity, recurrent financing, controlled configuration, maintainable integrations, documented releases, trained users, and measurable value. Scale decisions should consider readiness and service quality, not deployment volume alone.

Part VI. Reusable implementation assets

24. Country implementation package

Maintain one controlled package that links planning decisions to implementation evidence. The package may use documents, spreadsheets, repositories, and computable specifications, but ownership and versioning should remain clear.

Product foundation Vision, scope, roadmap, governance, decision log, prioritized backlog.	User and service design Personas, journeys, use cases, service workflows, job aids, and approvals.	Requirements Functional and non-functional requirements, acceptance criteria, traceability.	Technical design Architecture, FHIR package, terminology, configuration inventory, environment specification.
Interoperability Interface register, workflow specifications, mappings, test fixtures, and runbooks.	Assurance Test strategy, cases, results, security evidence, UAT, and residual-risk decisions.	Deployment Training, devices, data, communications, cutover, rollback, and go-live sign-off.	Operations Monitoring, support, incidents, access, backup, recovery, releases, costs, and improvement.

25. Core templates

Template	Primary purpose
Implementation readiness check	Confirm that planning outputs and ownership are sufficient to begin delivery.
Decision log	Record options, rationale, authority, affected assets, and review date.
Requirements traceability matrix	Connect user need, workflow, requirement, implementation asset, test, and status.
Workflow canvas	Describe trigger, actors, steps, decisions, records, tasks, exchange, and completion.
Architecture decision record	Record technical choice, context, options, consequences, and approval.
Configuration inventory	Track forms, terminology, settings, repositories, versions, environments, and owners.
Interface specification	Define outcome, source and destination, mappings, security, reliability, monitoring, and tests.
Test and acceptance record	Retain scenarios, expected results, evidence, defects, retest, and approval.
Go-live checklist	Confirm governance, content, technology, data, users, integration, operations, and cutover.
Operational runbook	Document monitoring, routine tasks, incidents, recovery, escalation, and service ownership.

26. Content ownership model

Program and clinical leads Own service workflows, content, decision rules, and service-quality measures.	Product owner Owns product vision, priorities, backlog, release scope, and acceptance.	Architecture and standards leads Own technical design, FHIR conformance, terminology patterns, and architecture decisions.
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Interoperability and data leads

Own interfaces, mapping, identity, exchange quality, warehouse models, and indicators.

Security and privacy leads

Own risk assessment, access principles, controls, review, and incident requirements.

Implementation and operations leads

Own readiness, training, rollout, support, monitoring, continuity, and handover.

27. Recommended companion resources

- Digital Community System Toolkit and its workshop and country-roadmap resources.
- Data.FI Reference eCHIS Implementation Guide and computable FHIR artifacts.
- Role-based community health worker, supervisor, administrator, and support guides.
- Reference architecture and infrastructure specification.
- Integration workflow repository, mapping specifications, test fixtures, and runbooks.
- Country adaptation workbook and implementation evidence repository.

Source base for this draft

This working draft consolidates and reorganizes the existing Data.FI DCS planning toolkit and current eCHIS implementation materials. Technical details should be checked against the controlled source assets before publication.

Source	Role in the guide
Digital Community System Toolkit (2024)	Person-centered planning, workshop design, ecosystem assessment, personas, use cases, functionality, governance, vision, monitoring, technical orientation, product selection, and country roadmap.
Data.FI Reference eCHIS Implementation Guide v0.1.0	FHIR design, template extraction, tasking, IPS alignment, selective synchronization, and conformance artifacts.
eCHIS Requirements Specification	Reference users, service modules, functional and non-functional requirements, reporting, implementation, and sustainability.
eCHIS user, supervisor, and administrator guides	Current role-based workflows, application behavior, administration, users, locations, organizations, and care teams.
eCHIS Mapping Specification and Solution Design documentation	Identity, referral, supply chain, surveillance, messaging, reporting, analytics, workflow assumptions, and mappings.
Infrastructure, security, and go-live materials	Reference platform needs, production hardening, test dependencies, launch evidence, monitoring, and handover.

Web reference: <https://palladium-group.github.io/datafi-echis-ig/>

Editorial note

For external publication, add approved acknowledgments, attribution, licensing, document governance, version history, repository links, and accessibility review. Replace internal working language and confirm all product names and technical identifiers against the release baseline.